

Women in EM Working Group – Gender Differences in the ED

Section 1: Case Summary

Scenario Title:	Allyship: Gendered Microaggressions
Date of Development:	March 18, 2021
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Revision Date (If applicable):	Jan 31, 2022
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Learners, Setting and Personnel			
Target Learners:	☐ Medical Students	☒ Junior Residents	☒ Senior Residents
	☒ Staff Physicians	☐ Inter-professional	☐ Other
Target Number of Learners:			
Location:	☐ Sim Lab	☐ In Situ	☒ Other
Personnel Required:	Instructors: 1-2		
	Sim Actors: 5		
	Sim Techs: 1		

Goals and Objectives	
Educational Goal:	Recognize gender bias and stereotypes that exist at multiple levels of the healthcare care team during resuscitations in the emergency department.
Objectives: (Medical and CRM)	- Identify examples of gender bias/stereotypes during a resuscitation scenario - Recognize situations in which team members can intervene to minimize or eliminate gender bias/stereotypes - Reflect on systems level structures that positively/negatively affect gender bias
EPAs Assessed:	

Case Summary: Brief Summary of Case Progression and Major Events
Telephone to ED: patient is en route ~ 5 min away Initial vitals: HR 112, BP 80/42, SpO2 94%, T 38.5, Glucose 6.3 ID: 60M with two days of increased confusion and urinary symptoms. Resident has ~3min to organize her team. Has 2 RNs (both males) available. Paramedic goes to male RN asks if he is the staff taking care of this patient (gender stereotype 1). RN informs paramedic that he is an RN and resident is leading this case. Resident and RN listen to handover together. Resuscitation begins. The patient is verbal but slightly confused. Comments on the resident's appearance "lucky me to get such a good-looking doctor" (gender stereotype 2). Once patient is more stabilized asks resident for warm blankets and ginger ale. Patient stabilizes and resident calls GIM. Consultant comes down and asks "who is in charge of this patient". Begins questioning the management and asks to speak to the staff (gender stereotype 3).



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Section 2: Scenario Requirements

A. Patient	
☒ Mannequin	☐ Standardized Patient
☐ Task Trainer	☐ Hybrid
B. Sim Actors	
Role	Description of role, expected behavior, any script required for actor (including conveying patient information if patient is unable)
RN 1 (M)	In charge of taking handover, meds and documentation. This is the RN that the patient always looks at when answering questions. Halfway through, this RN should direct the patient back towards the resident.
RN 2 (M or F)	Responsible for monitors, IV, etc. as designated by the resident.
Paramedic (F or M)	Asks resident if she is the nurse taking care of the patient before attempting to give handover. Leaves after giving handover.
Resident (F)	Resuscitation leader.
Consultant (F or M)	Comes down to see the patient and asks resident if she is the nurse looking after the patient.
C. Special Equipment Required	
Sim room sufficient – no additional equipment required	
D. Required Medications	
IVF Vasopressors – NE, E, phenylephrine, vasopressin Antibiotics – ceftriaxone, azithro, pip-tazo, vanco	
E. Moulage	
None required	

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Section 3: Baseline State

A. Clinical Vignette: Provided at Beginning of Case			
<i>Provided by nurse (just got a patch that...):</i> 60M from home with two days of increased confusion and urinary symptoms. Initial GCS 10, HR 112, BP 80/42 and temperature 38.5. Glucose 6.3.			
B. Patient Profile and History			
Patient Name: John		Age: 60	Weight: 70kg
Gender: ☒ M ☐ F		Code Status: Full	
Chief Complaint: Altered Mental Status			
History of Presenting Illness: 2 days of increased confusion and urinary symptoms			
Past Medical History:	BPH	Medications:	Tamsulosin
	HTN		Ramipril
	A fib		Metoprolol
			ASA
Allergies: None			
Social History: Lives in a bungalow in town with his wife. Retired school teacher. Normally independent in ADLs and IADLs. Over the past days, increasing forgetfulness and bizarre behaviour.			
Patient personality/background (thoughts, feelings, perspective): Answers questions in nonsense. Unable to provide a history. Just keeps saying "I feel unwell" or "I think so" to answer a question. Tangentially talks about something else in responses or makes inappropriate comments. After resident introduces self as doctor, patient to say "lucky me to get such a good looking doctor." Can also make comments like "are you old enough to be a doctor?"			
Review of Systems:	CNS:	Normal.	
	HEENT:	Normal.	
	CVS:	Normal.	
	RESP:	Normal.	
	GI:	Normal.	
	GU:	Dysuria, urgency, frequency x 4 days.	
	MSK:	Normal.	
C. Baseline Simulator State and Physical Exam			
☐ No Monitor Display		☒ Monitor On, no data displayed	
☐ Monitor on Standard Display			
HR: 112	BP: 80/42	RR: 14	O ₂ SAT: 94% RA
T: 38.5			
Rhythm: regular	Glucose: 6.3	GCS: 10	Pupils: pinpoint bilat
General Status:	Lying in bed, protecting airway, altered LOC.		
CNS:	Eyes open to verbal (3), incomprehensible responses (3), withdraws from pain (4)		
HEENT:	Normal.		
CVS:	Normal.		
RESP:	Normal		
ABDO:	Normal.		
GU:	Normal.		
MSK:	Normal.		
SKIN:	Normal.		



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Section 4: Scenario Progression

Scenario States, Modifiers and Triggers			
Patient State/Vitals	Patient Status	Learner Actions, Modifiers & Triggers to Move to Next State	
1. Handover Rhythm: irregularly irregular (known afib) HR: 112 BP: 80/42 RR: 14 O ₂ SAT: 94% RA T: 38.5	Verbal but somewhat confused. Cannot remember timeline etc. States hurts when voids but unable to answer other questions reliably.	<u>Gender Stereotype 1</u> ☐ Navigate first gender stereotype where male RN is assumed to be the staff <u>Medical Management</u> ☐ IV-O2-Monitors ☐ Initial assessment of the patient	<u>Modifiers</u> <i>Changes to patient condition based on learner action</i> <u>Triggers</u> <i>For progression to next state</i> - Handover complete and resident introduces self as doctor --> 2. Patient Management
2. Patient Management Rhythm: irregularly irregular HR: 112 BP: 80/42 RR: 14 O ₂ SAT: 94% RA T: 38.5	Unchanged	<u>Gender Stereotype 2</u> ☐ Navigate second gender stereotype when patient tells resident she is “good-looking” and asks for blankets/ginger ale <u>Medical Management</u> ☐ IVF ☐ Full set of labs and septic wu ☐ Antibiotics	<u>Modifiers</u> <u>Triggers</u> - IVF given, labs ordered, and Abx ordered --> 3. Improvement
3. Improvement Rhythm: irregularly irregular HR: 98 BP: 95/56 RR: 14 O ₂ SAT: 97% on NP/NRB T: 38.5	Unchanged	<u>Gender Stereotype 3</u> ☐ Navigate third gender stereotype when consultant questions management and asks to talk to the staff. When arrives in room, asks resident if she’s the nurse for the patient. <u>Medical Management</u> ☐ Handover to GIM	<u>Modifiers</u> - Give labs to learners at beginning of state <u>Triggers</u> - Discussion with GIM complete --> END CASE

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Section 5: Facilitator Cheat Sheet

Include key errors to watch for, common challenges with the case, and expected/common discussion topics. Supplemental information regarding any relevant pathophysiology, guidelines, or management information that may be reviewed during debriefing should be provided for facilitators to have as a reference.

This case aims to address microaggressions using simulation. Microaggressions are defined as daily, subtle snubs, slights, and insults directed towards historically stigmatized, oppressed or excluded groups that implicitly communicate hostility, general disrespect and devaluation. This case focusses specifically on microaggressions women commonly experience as health care providers.

This case, in many ways, serves as a launching point for conversation. **It is not a traditional simulation case;** instead of having the debriefing and discussion concentrate on medical management and individual participants' actions, facilitators intentionally describe the microaggressions that occurred and guide the discussion to focus on concrete actions that can be integrated into clinical practice. The case involves many actors. The role of the actors is essentially to commit the microaggressions toward the resident managing the case. **Because this case can be triggering, especially for individuals having experienced previous microaggressions, careful selection of actors and participants, as well as pre-briefing the resident leading the case far ahead of time are crucial.** When we ran this case with our residents, one of the case authors volunteered ahead of time to be the resident managing the case. This means she was aware beforehand that she would be having this experience. The other trainees watched the case.

We all know that simulation is a useful tool; it's impact and utility depend on who is using it and how well they know how to operate it. This is particularly true for this case. In addition to normal pre-briefing information about safe spaces, this case requires the explicit discussion of brave spaces. **It also requires that the facilitators have taken the time to learn about microaggressions and their impact.** For this session to have positive impact, the facilitators must also be versed in discussing ways that we can be allies to those experiencing microaggressions. The real value here is in educating each other through conversation. **A skilled facilitator is required to ensure biases are not perpetuated.** We would also recommend that the facilitators should be of diverse background – both those who have experienced microaggression (in this case, those identifying as cisgender women) and those who have not.

If you are interested in learning more about microaggressions and their impact on health care professionals, we recommend reading [this article by Samina Ali](#) or [this fantastic review article from Annals of Emergency Medicine](#).

For those looking to learn more about how to be an ally or how to respond when witnessing microaggressions, we would recommend resources such as [O3](#), [Open the Front Door](#), [the 5 D's or Call In/Call Out](#). **These resources are a bare minimum for facilitators looking to incorporate a session like this one into their curriculum.**

We hope that simulation-based sessions can help serve as a safe space for medical learners to develop a deeper understanding and a practical approach to Equity, Diversity, Inclusion, Indigeneity and Accessibility (EDIIA) within the medical world. We acknowledge that this session is but a starting point for conversations related to microaggressions and serves as a reminder of the ongoing work that is required to dismantle systems of oppression.



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Section 6: References

1. Torres MB, Salles A, Cochran A. Recognizing and Reacting to Microaggressions in Medicine and Surgery. Chicago: JAMA Surg; 2019 Sep. 1;154(9):868-872. [doi: 10.1001/jamasurg.2019.1648](https://doi.org/10.1001/jamasurg.2019.1648)
2. Ali, S. Be Polite. CJEM. 2022 Oct 5; 24:88-89. [doi: 10.1007/s43678-021-00208-6](https://doi.org/10.1007/s43678-021-00208-6)
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4. Tavares W, Eppich W, Cheng A, Miller S, Teunissen PW, Watling CJ, Sargeant J. Learning Conversations: An Analysis of the Theoretical Roots and Their Manifestations of Feedback and Debriefing in Medical Education. Acad Med; 2020 Jul. 95(7):1020-1025. [doi: 10.1097/ACM.0000000000002932](https://doi.org/10.1097/ACM.0000000000002932)
5. Interrupting Bias: Calling Out vs Calling In. Seed the Way; 2018. [cited 2022 April 26] Available from: http://www.racialequityvtnea.org/wp-content/uploads/2018/09/Interrupting-Bias_-Calling-Out-vs.-Calling-In-REVISED-Aug-2018-1.pdf



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Section 7: Laboratory Results

<u>CBC</u> WBC 25 Hgb 125 Plt 406 <u>Lytes</u> Na 135 K 4.0 Cl 120 HCO ₃ AG. 9 Urea Cr 252 Glucose 6.4 <u>Extended Lytes</u> NORMAL Ca Mg PO ₄ Albumin TSH <u>VBG</u> NORMAL pH. pCO ₂ pO ₂ HCO ₃ Lactate	<u>Cardiac/Coags</u> NORMAL Trop <10 D-dimer 252 INR 1.5 aPTT <u>Biliary</u> NORMAL AST ALT GGT ALP Bili Lipase <u>Tox</u> NEG EtOH ASA Tylenol Dig level Osmols <u>Other</u> UA: +nitrites, +leuks, ++blood
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Section 8: ECGs, X-rays, Ultrasounds and Pictures

Paste in any auxiliary files required for running the session. Don't forget to include their source so you can find them later!

ECG: normal
CXR: normal
FAST: negative
US IVC: collapsible
Urine cx: P
Blood cx: P

